

	EMERGENCY MEDICAL SERVICES AGENCY	POLICY NUMBER	724
	A Division of the Merced County Department of Public Health	Effective Date	11/2025
Authority: Health and Safety Code, Division 2.5, and California Code of Regulations, Title 22, Division 9, Chapter 4, 1797.220		Initial Date:	05/2011
		Review Date:	01/2028

ALTERED LEVEL OF CONSCIOUSNESS (ALOC) OR SYNCOPE	
ADULT	PEDIATRIC
BLS	
Assess Vitals / Obtain SpO ₂ / Oxygen - Titrate to SpO ₂ of 94% or higher / Secure Airway - Assist Ventilations if needed. Blood Sugar Check	
Blood glucose < 60 mg/dl: Oral Glucose - One (1) unit dose (15 gms) if the patient can swallow Recheck blood glucose after 3-5 minutes and repeat, as necessary	Blood glucose < 80 mg/dl: Oral Glucose - One (1) unit dose (15 gms) with an intact gag reflex (the patient can manage their own secretions) Recheck blood glucose after 3-5 minutes and repeat, as necessary
If opioid intoxication with respiratory depression (rate ≤ 8 per minute) is suspected, administer naloxone as follows:	
Naloxone - 2 mg IN (1 mg per nostril) <u>OR</u> FULL DOSE of prepackaged Naloxone IN in one nostril Repeat once after 3-5 minutes if there is no response	Naloxone < 20 kg, 1 mg IN (0.5 mg per nostril) ≥ 20 kg, 2 mg IN (1 mg per nostril) - May repeat after 3-5 minutes if there is no improvement or response
ALS	
Follow BLS procedures if applicable. Obtain 4-Lead ECG. Obtain 12-Lead EKG for any syncope or near syncopal episode (Adults Only). Vascular Access - IV/IO rate as appropriate.	
Blood glucose < 60 mg/d <u>D10 OR D50</u> : D10 - 15 gms (150 ml) to 25 gms (250 ml) IV/IO infusion D50 - 25 gms IV/IO Recheck blood glucose after 3-5 minutes and repeat as necessary	Blood glucose < 80 mg/dl <u>D10 OR D25</u> : D10 - 5 mL/kg - Max dose 25 gms (250 ml) D25 - 2 mL/kg - Max dose 25 gms - May repeat in 3-5 minutes if altered mental status persists and repeat blood glucose is

Signatures on File

Approved By: Tim Williams
EMS Administrator

Ajinder Singh, MD
EMS Medical Director

	<80 mg/dl
If No IV/IO Access:	
Glucagon – 1 mg IM - Recheck blood glucose after 3-5 minutes - May repeat once	Glucagon < 20 kg, 0.5 mg (0.5 mL) IM ≥ 20 kg, 1 mg IM - May repeat after 3-5 minutes if no improvement
If opioid intoxication with respiratory depression (rate ≤ 8 per minute) is suspected, administer naloxone as follows:	
Naloxone - 2 mg IN (1 mg per nostril) - Repeat IN dose after 3-5 minutes if no response If there is no response after the administration of 2 mg IN doses: 1 mg IV/IO/IM, titrated to increased respiratory rate or until a total of 10 mg of naloxone has been administered	Naloxone <20 kg, 1 mg IN (0.5 mg per nostril) ≥20 kg, 2 mg IN (1 mg per nostril) - Repeat after 3-5 minutes If there is no response after IN doses: 0.5 mg IV/IO/IM, titrated to increased respiratory rate or until a total of 8 mg of naloxone has been administered
Special Considerations	
Synthetic opioids (e.g. Fentanyl or Carfentanyl) may require higher doses of naloxone to restore respiratory effort. Naloxone MUST be administered before securing an airway if opioid intoxication is suspected. IN (intranasal) is the preferred route for Naloxone. Manage airway with aggressive BLS techniques (suction, airway adjuncts, BVM). If necessary, providers may use an approved supraglottic airway to secure the airway.	